

UNDERSTANDING DISABILITY

Introduction

It is estimated that one billion people, about 15% of people globally experience one or other form of disability which leads to adverse outcome. The prevalence is high when it comes to developing countries. Disability is a complex issue which is also multidimensional. Persons with disabilities may have varied needs and capacities, and they also have the same rights and equity like any other person in society. Too often their lives are handicapped by physical and social barriers in society which hamper their full participation. It is important to have a comprehensive understanding of disability and other related concepts. This unit therefore shall discuss these aspects along with types and causes of disability in the context of India.

Understanding Disability

Disability is a word used in daily conversations that holds different meanings for different people. The experience of disability is unique to each person but there are common impacting factors. Opinion on what constitutes disability may vary; considered as a medical problem, a sin or a curse, have to live with it etc. These views lead to how society accepts people with disabilities which unfortunately results in stigma and discrimination. This transition from an individual, medical perspective to a structural, social perspective has been described as the shift from a “medical model” to a “social model” in which people are viewed as being disabled by society rather than by their bodies.

A person with disability may be unable to perform certain functions as well as most other people. The disability may be physical; it may involve senses, including seeing or hearing; it may involve finding it difficult or impossible to think clearly; or it may involve mental health. Long term physical, mental, intellectual or sensory impairments may result in physical and social barriers that interferes with their functioning and participation in daily life, leading to a life of complete dependency in many cases. The following section provides explanation on what is disability and how it impairs one's functioning.

Definition

The World Health Organization (WHO) defines ‘Disability’ as “an umbrella term, covering impairments, activity limitations, and participation restrictions. Impairment is a problem in body function or structure; an activity limitation is a difficulty encountered by an individual in executing a task or action; while a participation restriction is a problem experienced by an individual in involvement in life situations. Thus, disability is a complex phenomenon, reflecting an interaction between features of a person's body and features of the society in which he or she lives.”

The Preamble to the Convention on the Rights of Persons with Disabilities (CRPD) -2006, adopted by the United Nations, describes disability as:

“Disability results from the interaction between persons with impairments and attitudinal and environmental barriers that hinder their full and effective participation in society on an equal basis with others.”

According to the Rights of Persons with Disabilities Act, 2016, "Person with disability" means a person with long term physical, mental, intellectual or sensory impairment which, in interaction with barriers, hinders his full and effective participation in society equally with others, and "Person with benchmark disability" means a person with not less than forty per cent. of a specified disability where specified disability has not been defined in measurable terms and includes a person with disability where specified disability has been defined in measurable terms, as certified by the certifying authority.

Concepts Related to Disability

Concepts of disability have evolved over the past few decades. The focus of attention has shifted from disability as a state, a consequence of disease, to disability being included as just one of many components of health. It is observed that many terms are used to refer to disability like “impairment,” “disability,” and “handicap,” that are often used interchangeably. But, when we examine what these terms actually mean, it is found that they have different connotations which are very important to be discussed so that it leads to a better understanding of the world of disability and how these terminologies are interconnected.

The most commonly cited definitions are those provided by the World Health Organization (1980) in The International Classification of Impairments, Disabilities, and Handicaps (known as ICF) which provides a conceptual framework for disability in three dimensions – impairment, disability and handicap.

Disease

Disease refers to any harm in the **functional state** of an organism that deviates it from the normal structure or function and is generally attributed with a set of signs and symptoms that differ from any physical injury or accident.

Impairment

It is any loss or abnormality of psychological, physiological or anatomical structure or function. It occurs at the **organ or system function**. For example: defective body part, amputated limb, paralysis after polio, limited hearing, etc.

Disability

Disability is any restriction or lack (resulting from an impairment) of ability to perform an activity in the manner or within the range considered normal for a human being. Disability is concerned with functional performance or activity, affecting the whole person. Impairment leads to disability in the sense that the person may have problem or difficulty in speaking, walking, hearing, or doing any such task due to any bodily impairment.

Handicap

Handicap is a disadvantage for a given individual, resulting from an impairment or a disability, that limits or prevents the fulfilment of a role that is normal (depending on age, sex, and social and cultural factors) for that individual. 'handicap'-focuses on the person as a social being and reflects the interaction with and adaptation to the person's surroundings. A disability becomes a handicap when it interferes with doing what is expected at a particular time in one's life.

It can be summarised as impairment referring to a problem with a structure or organ of the body; disability is a functional limitation with regard to a particular activity; and handicap refers to a disadvantage in filling a role in social life. A ‘person with disability’ therefore as per the Rights of Persons with Disabilities Act, 2016, means a person with long term physical, mental, intellectual or sensory impairment which, in interaction with barriers, hinders his full and effective participation in society equally with others.

Causes for Disability

Many factors can be attributed to disability and it can originate at any stage in a person's life, be it before birth, during or after birth, which are complex and heterogeneous. For a clear understanding, they can be classified as prenatal, perinatal and postnatal causes that are discussed below.

Prenatal Causes refer to those before birth. Genetic and chromosomal abnormalities play a significant role in causing disability that is inherited. X and Y chromosomal factor can lead to Down Syndrome.

Developmental malformation, maternal illness or infections (like German measles, syphilis, herpes, HIV), age and malnourished state of mother, any injury or fall of the pregnant mother; consanguineous marriage, any exposure to radiation or chemicals, ingestion of drugs, tobacco, alcohol and overuse of medicines by the mother during pregnancy etc. Lack of access to appropriate health during pregnancy due to poverty and other reasons can affect the child leading to low birth weight and other complications.

Perinatal Causes are those that occur during the birth process which are mostly biomedical causes. Babies born with low birth weight or premature are at great risk of developing congenital abnormalities. Complications during delivery, trauma during labour, poor treatment, lack of experts to handle the delivery and lack of essential facilities are other causes. Oxygen deprivation for the child during birth is another major reason that can lead to permanent disabilities in the child.

Postnatal Causes are those that happen after birth. Environmental factors play a major role here where accidents (motor vehicles, fire accidents), injuries (due to falls), conflicts, wars, risky work environments (mining, construction sites), breathing or drinking toxic chemicals etc are responsible for most of the disabilities in people. Illnesses such as meningitis, polio and measles during childhood can lead to permanent disability that lasts through life.

Types of Disabilities

Disability can be classified in many ways which varies across countries depending on their situation whether they are physical or mental or genetic etc. The International Classification of Functioning, Disability and Health (ICF) is the most current method of classifying disability as it uses a very neutral language and does not distinguish between the type and cause of disability. In India, The Right to Persons with Disabilities Act (2016) - RPWDA lists about 21 specified disabilities in the Schedule which are categorized into five major groups as presented in the section below.

Specified Disabilities as Mentioned in 'The Schedule of the RPWDA 2016'

S.No.	Categories	Specified Disabilities
1.	Physical Disability	1. Locomotor Disability
		2. Leprosy Cured Person
		3. Cerebral Palsy
		4. Dwarfism
		5. Muscular Dystrophy
		6. Acid Attack Victims
		7. Blindness
		8. Low Vision
		9. Deaf
		10. Hard of Hearing
		11. Speech and Language Disability
2.	Intellectual Disability	12. Intellectual Disability
		13. Specific Learning Disabilities
		14. Autism Spectrum Disorder
3.	Mental Behaviour	15. Mental Illness

4.	Disability due to Chronic Neurological Condition and Blood Disorder	16. Multiple Sclerosis
		17. Parkinson's Disease
		18. Haemophilia
		19. Thalassemia
		20. Sickle Cell Disease
5.	Multiple Disabilities	21. More than one of the above specific disabilities
6.	Any Other	

Brief description of each of these disabilities:

- i. **Locomotor disability** - inability to execute distinctive activities associated with movement of self and objects resulting from affliction of musculoskeletal or nervous system or both.
- ii. **Leprosy cured person** - a person who has been cured of leprosy but is suffering from loss of sensation in hands or feet, eye and eye-lid; extreme physical deformity as well as advanced age which prevents him/her from undertaking any gainful occupation.
- iii. **Cerebral palsy** - a group of non-progressive neurological condition affecting body movements and muscle coordination, caused by damage to one or more specific areas of the brain, usually occurring before, during or shortly after birth.
- iv. **Dwarfism** - a medical or genetic condition resulting in an adult height of 4 feet 10 inches (147 centimeters) or less.
- v. **Muscular dystrophy** - a group of hereditary genetic muscle disease that weakens the muscles that move the human body and persons with multiple dystrophies have incorrect and missing information in their genes, which prevents them from making the proteins they need for healthy muscles.
- vi. **Acid attack victims** - a person disfigured due to violent assaults by throwing of acid or similar corrosive substance.
- vii. **Blindness** - a condition where a person has total absence of sight; or visual acuity less than 3/60 or less than 10/200 (Snellen) in the better eye with best possible correction; or limitation of the field of vision subtending an angle of less than 10 degree.
- viii. **Low-vision** - a condition where a person has visual acuity not exceeding 6/18 or less than 20/60 up to 3/60 or up to 10/200 (Snellen) in the better eye with best possible corrections; or limitation of the field of vision subtending an angle of less than 40 degree up to 10 degree.
- ix. **Deaf** - persons having 70 DB hearing loss in speech frequencies in both ears.
- x. **Hard of hearing** - person having 60 DB to 70 DB hearing loss in speech frequencies in both ears.
- xi. **Speech and language disability** - a permanent disability arising out of conditions such as laryngectomy or aphasia affecting one or more components of speech and language due to organic or neurological causes.
- xii. **Intellectual disability** - a condition characterised by significant limitation both in intellectual functioning (reasoning, learning, problem solving) and in adaptive behaviour which covers a range of every day, social and practical skills.

- xiii. **Specific learning disabilities** - a heterogeneous group of conditions wherein there is a deficit in processing language, spoken or written, that may manifest itself as a difficulty to comprehend, speak, read, write, spell, or to do mathematical calculations and includes such conditions as perceptual disabilities, dyslexia, dysgraphia, dyscalculia, dyspraxia and developmental aphasia.
- xiv. **Autism spectrum disorder** - a neuro-developmental condition typically appearing in the first three years of life that significantly affects a person's ability to communicate, understand relationships and relate to others, and is frequently associated with unusual or stereotypical rituals or behaviours.
- xv. **Mental illness** - a substantial disorder of thinking, mood, perception, orientation or memory that grossly impairs judgment, behaviour, capacity to recognise reality or ability to meet the ordinary demands of life, but does not include retardation.
- xvi. **Multiple sclerosis** - an inflammatory, nervous system disease in which the myelin sheaths around the axons of nerve cells of the brain and spinal cord are damaged, leading to demyelination and affecting the ability of nerve cells in the brain and spinal cord to communicate with each other.
- xvii. **Parkinson's disease** - a progressive disease of the nervous system marked by tremor, muscular rigidity, and slow, imprecise movement, chiefly affecting middle-aged and elderly people associated with degeneration of the basal ganglia of the brain and a deficiency of the neurotransmitter dopamine.
- xviii. **Haemophilia** - an inheritable disease, usually affecting only male but transmitted by women to their male children, characterised by loss or impairment of the normal clotting ability of blood so that a minor may result in fatal bleeding.
- xix. **Thalassemia** - a group of inherited disorders characterised by reduced or absent amounts of haemoglobin.
- xx. **Sickle cell disease** - a haemolytic disorder characterised by chronic anaemia, painful events, and various complications due to associated tissue and organ damage; "haemolytic" refers to the destruction of the cell membrane of red blood cells resulting in the release of haemoglobin.
- xxi. **Multiple Disabilities** (more than one of the above specified disabilities).

Any other category as may be notified by the Central Government can be considered as disability.

PWD Population in India

The WHO estimated that more than six hundred million people across the globe live with disabilities of various types due to chronic diseases, injuries, violence, infectious diseases, malnutrition, and other causes related to poverty. In India, about 2.68 Cr (26.8 million) persons are 'disabled' which is 2.21% of the total population (Census, 2011). There has been a marginal increase in the differently-abled population in India, with the figure rising from 21.9 million in 2001 to 26.8 million over the period of 10 years. Among the disabled population, 56% were males and 44% were females. As per the Census 2011, there are 14.9 million men with disabilities, 11.9 million women and 2.04 million children with disabilities in the country. The total number of differently-abled people is over 18.0 million in the rural areas and just 8.1 million in the urban settings.

Visual impairment is highest at 48.5% followed by movement (27.9%), mental illness (10.3%), speech impairment (7.5%) and hearing loss (5.8%). Highest number of disabled is reported in the state of Uttar Pradesh (3.6 million), followed by Bihar, West Bengal, Tamil Nadu and Maharashtra. As for employment data, only 36.3% of the disabled are employed (about 98 lakh – 71 lakh men and 27 lakh women). 45% of women with disabilities are literate compared to 62% men who are literate. These statistics are based on 2011 Census where only eight types of disabilities were part of the survey. This

was collected before the implementation of the Rights of Persons with Disabilities Act 2016 which recognizes 21 conditions as disabilities. Hence, it is hoped that more accurate number of people with disabilities will be included in that.

Changing Types of Disability

As the concept of disability is constantly evolving, so also the types. It has been increasing over the years and many forms of disabilities get included in the list approved by the Central Government. In the Persons with Disabilities (Equal Opportunities, Protection of Rights and Full Participation) Act, 1995, only seven types of disabilities were included. But with the latest Rights of Persons with Disabilities Act, 2016, it has extended covering 21 types of disabilities. The new list of recognized disabilities includes three blood disorders (Thalassemia, Haemophilia and Sickle Cell disease) and acid attack survivors. Intellectual disability, Parkinson's disease, Cerebral Palsy, Dwarfism and Autistic Spectrum disorders are also included in the list.

This change in types of disabilities is reflected in the disabled population census. Question on disability was included in all the censuses of India since 1872 till 1931. It was not part of the census data from 1941 to 1971, and then in 1981 census, data related to three types of disability (totally blind, totally crippled and totally dumb) alone was collected. In 1991, disability data was not collected and in 2001, only five types of disability numbers (disability in seeing, speech, hearing, moving and mental disability) were gathered. This further extended to eight types in 2011 census where mental retardation, any other and multiple disability was included to the existing list. Different kinds of disability in the Act are classified based on medical grounds and not on social perception of disability.

With different types of disabilities being included, it is envisaged that people with disabilities will be able to have access to all the rights, opportunities and privileges enshrined in the national and international conventions that enable them lead a qualitative life and maximise their full potential.